

Title

Feasibility of Comparing Empirical Antibiotic Regimens in Septic Intensive-Care Patients: A Multi-Centre Adaptive Trial Evaluation

Abstract

The feasibility phase enrolled 200 of 284 screened critically ill patients with sepsis across 10 intensive care units over a 5.5-month period. Participant demographic characteristics showed a median age around 70 years, with approximately two-thirds males and over 40% on mechanical ventilation at baseline. Four of five feasibility criteria were met: recruitment achieved 70% of screened patients, protocol adherence exceeded 80%, data availability for the primary outcome approached 86% by trial end (though initially lagged in early months), and voluntary data withdrawal remained below 3%. The unmet criterion regarding timely outcome ascertainment improved substantially in the final month, approaching target thresholds. Thirty-day mortality was observed in approximately 30% of randomised participants. Serious adverse reactions attributable to study medications occurred in a small minority. These results support trial feasibility with a modification focusing on outcome data capture processes. The randomised design, eligibility criteria, and measurement approaches proved workable in routine intensive care settings. Investigators concluded that a full-scale definitive trial comparing these empirical antibiotics is justified.